NUR 774 Cardiac Addendum: 100-Question Test

33 Select All That Apply + 67 single answer, grouped by topic. Each question is followed by its answer, rationale, and a mnemonic. Includes the earlier 52 addendum questions plus 48 new, harder ones on nursing care, medications, and monitoring.

> Heads-up: the addendum's Kawasaki slide says ibuprofen; your integrated study guide and current AHA guidance say ASPIRIN + IVIG. Ask your professor which they want.

How to use: answer the question, then swipe to the next page for the answer, rationale, and mnemonic.

SINGLE ANSWER

Which history finding in a 2-month-old most suggests a heart problem?

A. Six wet diapers a day

B. Spitting up small amounts after feeds

C. Sleeping about 16 hours a day

D. Sweating and tiring during feedings

Swipe for answer →

Answer 1

D

- **Sweating and tiring during feedings**

Why: Feeding is hard work for an infant. Diaphoresis, fatigue, and long feeds signal heart failure. The others are normal infant findings.

> 🧠 **"Feeding is the infant's treadmill." Sweaty, tired feeder = check the heart.**

SELECT ALL THAT APPLY

Which tests are used to assess heart disorders in children?

A. Tympanometry

B. Lumbar puncture

C. Exercise testing

D. Echocardiography

E. Electrocardiogram (ECG)

F. Cardiac MRI

G. Chest radiography

Swipe for answer →

Answer 2

C, D, E, F, G

- **Exercise testing**
- **Echocardiography**
- **Electrocardiogram (ECG)**
- **Cardiac MRI**
- **Chest radiography**

Why: The addendum lists ECG, radiography, echocardiography, phonocardiography/MRI, exercise testing, and labs.

> 🧠 **"Every Child's Heart Exam Matters": ECG, Chest x-ray, Heart echo, Exercise test, MRI.**

SINGLE ANSWER

Which test best shows a hole between the heart chambers?

A. Electrocardiogram

B. Pulse oximetry

C. Echocardiogram

D. Phonocardiography

Swipe for answer →

Answer 3

C

- **Echocardiogram**

Why: Echo shows anatomy, valves, and blood flow direction. An ECG shows electrical activity only.

> 🧠 **"Echo = Eyes (sees the anatomy). ECG = Electricity."**

SELECT ALL THAT APPLY

A nurse is doing a physical assessment on a child with a suspected heart defect. Which should be included?

A. Blood pressure

B. General appearance and skin color

C. Respiratory rate and effort

D. Pulses in the arms AND legs

E. Visual acuity

F. Tympanic membrane mobility

Swipe for answer →

Answer 4

A, B, C, D

- **Blood pressure**
- **General appearance and skin color**
- **Respiratory rate and effort**
- **Pulses in the arms AND legs**

Why: Cardiac physical assessment = general appearance (color, cyanosis, growth), pulses, BP, and respirations.

> 🧠 **"Look, Feel, Measure": Look (color, appearance), Feel (pulses in all 4 limbs), Measure (BP, HR, RR).**

SINGLE ANSWER

A parent asks what an echocardiogram will be like for their 3-year-old. Which response is best?

A. "Your child will need an IV dye and must fast overnight."

B. "A small tube will be threaded through the groin."

C. "It uses sound waves, doesn't hurt, and your child will need to lie still."

D. "It uses a lot of radiation, so you'll need to leave the room."

Swipe for answer →

Answer 5

C

- **"It uses sound waves, doesn't hurt, and your child will need to lie still."**

Why: Echo is noninvasive ultrasound with no radiation. Lying still is the main challenge, and young children may need sedation. A catheter through the groin is a cardiac cath.

> 🧠 **"Echo = Easy: sound waves, no needles, no rays."**

SELECT ALL THAT APPLY

Which nursing actions are appropriate AFTER a cardiac catheterization?

A. Check pulses below the insertion site

B. Bend the leg to prevent stiffness

C. Compare temperature and color of both legs

D. Check the insertion site for bleeding

E. Keep the affected leg straight as ordered

F. Encourage walking right away

Swipe for answer →

Answer 6

A, C, D, E

- **Check pulses below the insertion site**
- **Compare temperature and color of both legs**
- **Check the insertion site for bleeding**
- **Keep the affected leg straight as ordered**

Why: Bleeding and loss of circulation to the leg are the key risks. The leg stays straight for several hours as ordered.

> 🧠 **"Site, Pulse, Color, Warmth, and keep it Straight."**

SINGLE ANSWER

The nurse finds bleeding at a child's femoral cath site. What is the FIRST action?

A. Call the parents

B. Remove the dressing to look

C. Elevate the head of the bed

D. Apply direct, continuous pressure just above the insertion site

Swipe for answer →

Answer 7

D

- **Apply direct, continuous pressure just above the insertion site**

Why: Direct pressure (about 1 inch above the percutaneous site) stops arterial bleeding. Then notify the provider.

> 🧠 **"Bleeding? Press first, page second."**

SELECT ALL THAT APPLY

Which history findings suggest heart disease in a child?

A. Exercise intolerance

B. Family history of heart defects or sudden death

C. Early tooth eruption

D. Poor weight gain

E. Curly hair

F. Tiring or sweating with feeds

G. Frequent respiratory infections

Swipe for answer →

Answer 8

A, B, D, F, G

- **Exercise intolerance**
- **Family history of heart defects or sudden death**
- **Poor weight gain**
- **Tiring or sweating with feeds**
- **Frequent respiratory infections**

Why: Growth, feeding, infections, activity tolerance, and family history are core parts of the cardiac history.

> 🧠 **"Grow, Eat, Breathe, Play, Family."**

SINGLE ANSWER

What is the purpose of exercise testing in a child with a heart condition?

A. To see how the heart rhythm, blood pressure, and endurance respond to exercise

B. To diagnose a septal defect

C. To check for strep

D. To measure lung volumes

Swipe for answer →

Answer 9

A

- **To see how the heart rhythm, blood pressure, and endurance respond to exercise**

Why: Exercise testing (treadmill or bike) checks the heart's response to effort and exercise capacity.

> 🧠 **"Treadmill test = how the heart handles hustle."**

SINGLE ANSWER

Symptoms of a large VSD usually appear at what age?

A. The first hour of life

B. 6 months

C. 4–8 weeks

D. School age

Swipe for answer →

Answer 10

C

- **4–8 weeks**

Why: Pulmonary resistance falls over the first weeks, so more blood shunts left → right and the infant becomes symptomatic around 4–8 weeks.

> 🧠 **"4 to 8, the lungs relax and the VSD floods them."**

SINGLE ANSWER

Which murmur is classic for a VSD?

A. Soft musical murmur that is louder lying down

B. Fixed split S2

C. Harsh holosystolic murmur at the lower left sternal border

D. Continuous machinery murmur

Swipe for answer →

Answer 11

C

- **Harsh holosystolic murmur at the lower left sternal border**

Why: The addendum says "fatigue and harsh murmur." Machinery = PDA; fixed split = ASD; soft musical = Still's (innocent).

> 🧠 **"VSD = Very loud." Small hole, loud howl.**

SELECT ALL THAT APPLY

Which statements about a ventricular septal defect are correct?

A. It causes cyanosis at birth from right-to-left flow

B. Many small VSDs close on their own

C. VSDs that do not close may need surgery

D. It always needs emergency prostaglandin E1

E. It increases pulmonary blood flow

F. It is an opening between the ventricles

Swipe for answer →

Answer 12

B, C, E, F

- **Many small VSDs close on their own**
- **VSDs that do not close may need surgery**
- **It increases pulmonary blood flow**
- **It is an opening between the ventricles**

Why: A VSD is a left-to-right (acyanotic) shunt. Most small ones close spontaneously; persistent significant ones need surgical closure.

> 🧠 **"Small hole: wait and watch. Big hole: fix it."**

SINGLE ANSWER

A parent asks if their infant's small VSD will need surgery. Which response is best?

A. "VSDs never close without surgery."

B. "All VSDs need open-heart surgery right away."

C. "The loud murmur means it is a large, dangerous hole."

D. "Many small VSDs close on their own; the cardiologist will follow it."

Swipe for answer →

Answer 13

D

- **"Many small VSDs close on their own; the cardiologist will follow it."**

Why: Most small VSDs close spontaneously. A small VSD can be LOUD; loudness does not equal severity.

> 🧠 **"Small hole, loud howl, often closes."**

SINGLE ANSWER

A 6-week-old with a large VSD tires during feeds and is not gaining weight. Which plan is best?

A. Stop feeds and give IV fluids only

B. Feed large volumes every 6 hours

C. Dilute the formula to reduce fluid

D. High-calorie formula (24–30 kcal/oz) in small, frequent feeds, with NG feeding if the infant tires

Swipe for answer →

Answer 14

D

- **High-calorie formula (24–30 kcal/oz) in small, frequent feeds, with NG feeding if the infant tires**

Why: Infants in heart failure burn more calories and tire easily. Pack in calories without adding volume.

> 🧠 **"Pack the calories, not the volume: 24-27-30."**

SELECT ALL THAT APPLY

Which findings would the nurse expect in a 6-week-old with a LARGE VSD?

A. Cyanosis at birth

B. Bradycardia

C. Tachypnea

D. Hepatomegaly

E. Sweating during feeds

F. Poor weight gain

Swipe for answer →

Answer 15

C, D, E, F

- **Tachypnea**
- **Hepatomegaly**
- **Sweating during feeds**
- **Poor weight gain**

Why: As lung resistance falls, extra blood floods the lungs → heart failure at about 4–8 weeks. VSD is acyanotic.

> 🧠 **"Big VSD at 6 weeks: Fast breathing, Sweaty, Skinny, Swollen liver."**

SINGLE ANSWER

A child with a large VSD was never repaired and now has cyanosis. What has most likely happened?

A. The VSD closed on its own

B. The ductus arteriosus reopened

C. Pulmonary hypertension has reversed the shunt to right-to-left (Eisenmenger)

D. The child developed asthma

Swipe for answer →

Answer 16

C

- **Pulmonary hypertension has reversed the shunt to right-to-left (Eisenmenger)**

Why: Years of extra lung flow damage the lung vessels → pulmonary HTN → right-sided pressure exceeds left → blood shunts right-to-left → cyanosis.

> 🧠 **"Flooded lungs Fight back → Flip the shunt."**

SINGLE ANSWER

After surgical VSD repair with a patch, how long is endocarditis prophylaxis needed (assuming no residual leak)?

A. 6 months after repair

B. Lifelong

C. Only until discharge

D. Never

Swipe for answer →

Answer 17

A

- **6 months after repair**

Why: Prosthetic material needs about 6 months to be covered by heart tissue. A residual leak next to the patch would mean ongoing prophylaxis.

> 🧠 **"Patch needs 6 months to heal over."**

SINGLE ANSWER

Which heart sound is the classic sign of an atrial septal defect?

A. Fixed splitting of the second heart sound (S2)

B. Harsh holosystolic murmur

C. Diastolic murmur at the apex

D. Continuous machinery murmur

Swipe for answer →

Answer 18

A

- **Fixed splitting of the second heart sound (S2)**

Why: Extra right-sided flow delays pulmonic valve closure, so S2 is widely split and doesn't change with breathing.

> 🧠 **"ASD = Always Split, Doesn't change."**

SINGLE ANSWER

When is an ASD often discovered?

A. Only after a stroke

B. At birth because of severe cyanosis

C. Later in childhood, in a child without symptoms

D. At 4–8 weeks with heart failure

Swipe for answer →

Answer 19

C

- **Later in childhood, in a child without symptoms**

Why: Many children with an ASD have no symptoms; it's found later from a murmur or split S2.

> 🧠 **"ASD = A Silent Defect."**

SINGLE ANSWER

A child had an ASD closed with a device in the cath lab. Which medication teaching is expected?

A. Digoxin for life

B. No medicines are ever used

C. Low-dose aspirin for about 6 months

D. Warfarin for life

Swipe for answer →

Answer 20

C

- **Low-dose aspirin for about 6 months**

Why: Low-dose aspirin prevents clots on the device while heart tissue grows over it.

> 🧠 **"New device? Aspirin for 6 months."**

SINGLE ANSWER

The parent of a 4-year-old with a small ASD asks about soccer. Which response is correct?

A. "Only non-contact sports, and only with oxygen."

B. "No exercise restriction is needed."

C. "No sports until age 18."

D. "Sports are allowed only after surgery."

Swipe for answer →

Answer 21

B

- **"No exercise restriction is needed."**

Why: Chapter 38: exercise restriction is unnecessary with an ASD.

> 🧠 **"ASD = A Sporty Defect: let them play."**

SELECT ALL THAT APPLY

Which are complications of an untreated large ASD later in life?

A. A clot crossing to the body (paradoxical embolus)

B. Pulmonary hypertension

C. Right ventricular enlargement and failure

D. Atrial arrhythmias

E. Left ventricular outflow obstruction

F. Coronary artery aneurysm

Swipe for answer →

Answer 22

A, B, C, D

- **A clot crossing to the body (paradoxical embolus)**
- **Pulmonary hypertension**
- **Right ventricular enlargement and failure**
- **Atrial arrhythmias**

Why: Extra right-sided volume wears out the right heart; clots can cross the hole into the systemic circulation.

> 🧠 **"Right side wears out, clots sneak across."**

SELECT ALL THAT APPLY

Which defects INCREASE pulmonary blood flow?

A. Tricuspid atresia

B. Patent ductus arteriosus

C. Tetralogy of Fallot

D. Ventricular septal defect

E. Atrial septal defect

F. Pulmonic stenosis

Swipe for answer →

Answer 23

B, D, E

- **Patent ductus arteriosus**
- **Ventricular septal defect**
- **Atrial septal defect**

Why: Left-to-right shunts send extra blood to the lungs. TOF and tricuspid atresia DECREASE pulmonary flow; PS is obstructive.

> 🧠 **"The D's flood the lungs": ASD, VSD, PDA all end in D.**

SINGLE ANSWER

The ductus arteriosus connects which two structures?

A. Pulmonary artery and aorta

B. Umbilical vein and inferior vena cava

C. Right and left ventricles

D. Right and left atria

Swipe for answer →

Answer 24

A

- **Pulmonary artery and aorta**

Why: In the fetus it lets blood skip the lungs. It should start closing with the first breaths (the addendum says closure is complete by about 3 months).

> 🧠 **"Ductus ARTERIOSUS connects the two big ARTERIES."**

SELECT ALL THAT APPLY

Which findings are expected with a significant PDA?

A. Cyanosis at birth

B. Wide pulse pressure

C. Bounding pulses

D. Continuous machinery murmur

E. Fixed split S2

F. Weak femoral pulses with high arm blood pressure

Swipe for answer →

Answer 25

B, C, D

- **Wide pulse pressure**
- **Bounding pulses**
- **Continuous machinery murmur**

Why: Blood runs aorta → pulmonary artery during systole AND diastole, which makes the continuous murmur and wide pulse pressure.

> 🧠 **"PDA = Machinery, Wide, Bounding" ("Mighty Wide Bounce").**

SINGLE ANSWER

A premature infant has a symptomatic PDA. Which medication does the nurse anticipate?

A. Indomethacin

B. Prostaglandin E1

C. Furosemide only

D. Digoxin

Swipe for answer →

Answer 26

A

- **Indomethacin**

Why: Indomethacin (or ibuprofen) blocks prostaglandin and helps the ductus close in preemies.

> 🧠 **"Indomethacin Interrupts (closes) it. Prostaglandin keeps it Patent (open)."**

SELECT ALL THAT APPLY

Which are management options for a PDA?

A. Prostaglandin E1 to close it

B. Closure during cardiac catheterization

C. Antibiotics to dissolve it

D. Indomethacin in selected premature infants

E. Surgical ligation

Swipe for answer →

Answer 27

B, D, E

- **Closure during cardiac catheterization**
- **Indomethacin in selected premature infants**
- **Surgical ligation**

Why: The addendum lists indomethacin, cardiac catheterization, and surgery. PGE1 does the OPPOSITE: it keeps the ductus open.

> 🧠 **"Pill, Plug, or Procedure": Indomethacin (pill), cath device (plug), surgery (procedure).**

SINGLE ANSWER

A premature infant is receiving indomethacin for a PDA. Which assessment is the priority?

A. Bowel sounds only

B. Hearing

C. Pupil size

D. Urine output (renal effects)

Swipe for answer →

Answer 28

D

- **Urine output (renal effects)**

Why: Indomethacin can decrease kidney blood flow (low urine output) and affect bleeding and the GI tract. Monitor urine output closely.

> 🧠 **"Indomethacin = Watch the Pee."**

SINGLE ANSWER

Why does a PDA increase blood flow to the lungs after birth?

A. Pulmonary pressure becomes higher than aortic pressure

B. Aortic pressure is higher than pulmonary pressure, so blood flows aorta → pulmonary artery

C. The foramen ovale stays open

D. Blood flows from the right atrium to the left atrium

Swipe for answer →

Answer 29

B

- **Aortic pressure is higher than pulmonary pressure, so blood flows aorta → pulmonary artery**

Why: As lung resistance falls after birth, blood flows from the high-pressure aorta into the pulmonary artery and recirculates through the lungs.

> 🧠 **"High to Low: Aorta pushes into the lungs."**

SELECT ALL THAT APPLY

Which findings suggest a significant PDA in a premature infant?

A. Bounding pulses

B. Increasing oxygen needs

C. Tachypnea

D. Narrow pulse pressure

E. Wide pulse pressure

F. Weak, thready pulses

Swipe for answer →

Answer 30

A, B, C, E

- **Bounding pulses**
- **Increasing oxygen needs**
- **Tachypnea**
- **Wide pulse pressure**

Why: Runoff from the aorta into the pulmonary artery causes bounding pulses and a wide pulse pressure; lung overflow causes respiratory distress.

> 🧠 **"PDA = Bounce, Wide, Breathless."**

SINGLE ANSWER

A healthy 4-month-old born at TERM still has a small PDA. Which plan is expected?

A. Follow for closure in the cath lab (device) or surgery; indomethacin won't work

B. No follow-up needed

C. Indomethacin now

D. Prostaglandin E1 infusion

Swipe for answer →

Answer 31

A

- **Follow for closure in the cath lab (device) or surgery; indomethacin won't work**

Why: Indomethacin/ibuprofen only works in preemies. Term infants have device (coil/plug) closure or surgical ligation.

> 🧠 **"Indomethacin is for Infants born early only."**

SINGLE ANSWER

Pulmonic stenosis causes hypertrophy of which chamber?

A. Left atrium

B. Right ventricle

C. Right atrium only

D. Left ventricle

Swipe for answer →

Answer 32

B

- **Right ventricle**

Why: The right ventricle must push blood through a narrow pulmonary valve, so it thickens.

> 🧠 **"Pulmonic = Pushes the Right side."**

SELECT ALL THAT APPLY

Which findings are associated with pulmonic stenosis?

A. Signs of right-sided heart failure

B. Systolic ejection murmur

C. Bounding pulses

D. Cyanosis when severe

E. Right ventricular hypertrophy

F. Continuous machinery murmur

Swipe for answer →

Answer 33

A, B, D, E

- **Signs of right-sided heart failure**
- **Systolic ejection murmur**
- **Cyanosis when severe**
- **Right ventricular hypertrophy**

Why: The addendum: mild right-sided heart failure, cyanosis, systolic ejection murmur (SEM). Critical PS can limit lung blood flow, causing cyanosis.

> 🧠 **"PS = Pressure on the Right, Systolic murmur."**

SINGLE ANSWER

What is the usual treatment to relieve pulmonic stenosis?

A. Bed rest only

B. Indomethacin

C. Balloon angioplasty/valvuloplasty

D. Lifelong prostaglandin E1

Swipe for answer →

Answer 34

C

- **Balloon angioplasty/valvuloplasty**

Why: A balloon catheter stretches the narrow valve open.

> 🧠 **"Narrow valve? Balloon it open."**

SINGLE ANSWER

A newborn with CRITICAL pulmonic stenosis is cyanotic. Why?

A. The newborn has pneumonia

B. The aorta is narrowed

C. Blood shunts left-to-right through a VSD

D. Blood shunts right-to-left across the foramen ovale

Swipe for answer →

Answer 35

D

- **Blood shunts right-to-left across the foramen ovale**

Why: High right-sided pressure pushes blood through the foramen ovale to the left side, skipping the lungs → cyanosis.

> 🧠 **"Right side Blocked → blood takes the Back door (foramen ovale)."**

SINGLE ANSWER

A child with pulmonic stenosis also has short stature and wide-set eyes. Which syndrome should the nurse suspect?

A. Kawasaki disease

B. Noonan syndrome

C. Marfan syndrome

D. Down syndrome

Swipe for answer →

Answer 36

B

- **Noonan syndrome**

Why: Noonan syndrome: valvular pulmonic stenosis and HCM, with short stature.

> 🧠 **"Noonan = PS + short Nursery-sized stature."**

SELECT ALL THAT APPLY

Severe pulmonic stenosis increases right ventricular pressure. Which findings can result?

A. Pulmonary edema as the first sign

B. Signs of right-sided heart failure (hepatomegaly, edema)

C. Cyanosis if severe

D. Right ventricular hypertrophy

E. Left ventricular hypertrophy

Swipe for answer →

Answer 37

B, C, D

- **Signs of right-sided heart failure (hepatomegaly, edema)**
- **Cyanosis if severe**
- **Right ventricular hypertrophy**

Why: The right side works harder → RVH → right-sided failure (fluid backs up into the body). Pulmonary edema is left-sided.

> 🧠 **"PS = Pressure on the Right → Right-sided failure."**

SINGLE ANSWER

Aortic stenosis leads to hypertrophy of which chamber?

A. Right ventricle

B. Right atrium

C. Left atrium only

D. Left ventricle

Swipe for answer →

Answer 38

D

- **Left ventricle**

Why: The left ventricle must push blood through a narrow aortic valve.

> 🧠 **"Aortic = Left (A comes Last, on the Left side)." Pulmonic = Right.**

SELECT ALL THAT APPLY

Which are concerns in a child with aortic stenosis?

A. Continuous machinery murmur

B. High arm BP with weak femoral pulses

C. Chest pain with exertion

D. Fainting (syncope)

E. A heart murmur

F. Risk of sudden death

Swipe for answer →

Answer 39

C, D, E, F

- **Chest pain with exertion**
- **Fainting (syncope)**
- **A heart murmur**
- **Risk of sudden death**

Why: The addendum: usually no symptoms except a murmur, but chest pain and even sudden death can occur. Exertional symptoms are red flags.

> 🧠 **"AS = Avoid Sudden exertion": chest pain, syncope, sudden death.**

SINGLE ANSWER

A teen with known aortic stenosis wants to join the basketball team. Which response is best?

A. "You need a cardiology evaluation before being cleared for sports."

B. "Only avoid sports if you get a fever."

C. "Just drink extra water during games."

D. "Sports are fine since you have no symptoms."

Swipe for answer →

Answer 40

A

- **"You need a cardiology evaluation before being cleared for sports."**

Why: Exertion can cause chest pain, syncope, or sudden death in AS. Sports clearance must come from cardiology.

> 🧠 **"Faints while running = Referral."**

SELECT ALL THAT APPLY

Which are treatment options for aortic stenosis?

A. Balloon valvuloplasty

B. Valve replacement

C. Indomethacin

D. Prostaglandin E1 for long-term treatment

E. Beta blocker or calcium channel blocker for stabilization

Swipe for answer →

Answer 41

A, B, E

- **Balloon valvuloplasty**
- **Valve replacement**
- **Beta blocker or calcium channel blocker for stabilization**

Why: The addendum: stabilization with a beta blocker or CCB, balloon valvuloplasty, or valve replacement.

> 🧠 **"Calm it, Balloon it, or Replace it."**

SINGLE ANSWER

Which blood pressure finding is typical of aortic stenosis?

A. Higher BP in the legs than the arms

B. Wide pulse pressure

C. Very high diastolic BP only

D. Narrow pulse pressure

Swipe for answer →

Answer 42

D

- **Narrow pulse pressure**

Why: Limited outflow through the narrow valve makes a small difference between systolic and diastolic BP. Wide pulse pressure is PDA.

> 🧠 **"AS = A Squeezed pulse pressure."**

SINGLE ANSWER

A newborn with critical aortic stenosis is most at risk for:

A. Polycythemia

B. Hypertension in the legs

C. Endocarditis in the first hour

D. Low cardiac output and shock

Swipe for answer →

Answer 43

D

- **Low cardiac output and shock**

Why: Critical AS blocks blood leaving the left ventricle → heart failure, low output, shock. It needs urgent balloon valvuloplasty.

> 🧠 **"Critical AS = the Exit is blocked → shock."**

SINGLE ANSWER

A teen with aortic stenosis is started on a beta blocker. Which teaching is correct?

A. "Don't stop it suddenly, and report dizziness or a slow pulse."

B. "Stop it if you feel tired."

C. "Take it only when you have chest pain."

D. "Double the dose before sports."

Swipe for answer →

Answer 44

A

- **"Don't stop it suddenly, and report dizziness or a slow pulse."**

Why: Beta blockers slow the heart and reduce workload. Stopping abruptly can cause rebound tachycardia.

> 🧠 **"Beta = Be steady: no sudden stops."**

SELECT ALL THAT APPLY

A child with aortic stenosis should seek immediate evaluation for which symptoms?

A. Palpitations

B. Chest pain with exercise

C. Dizziness

D. Fainting

E. Runny nose

F. Mild sunburn

Swipe for answer →

Answer 45

A, B, C, D

- **Palpitations**
- **Chest pain with exercise**
- **Dizziness**
- **Fainting**

Why: Exertional chest pain and syncope can come before sudden death in AS.

> 🧠 **"AS = Avoid Sudden collapse: Chest pain, Faint, Dizzy, Flutters."**

SINGLE ANSWER

Which finding is classic for coarctation of the aorta?

A. Higher blood pressure in the arms than the legs

B. Higher blood pressure in the legs than the arms

C. Fixed split S2

D. Continuous machinery murmur

Swipe for answer →

Answer 46

A

- **Higher blood pressure in the arms than the legs**

Why: The narrowing blocks flow to the lower body: pressure is high above it and low below it, with weak or absent femoral pulses.

> 🧠 **"Strong upstairs, weak downstairs."**

SELECT ALL THAT APPLY

Which symptoms may an older child with coarctation of the aorta have?

A. Nosebleeds

B. Leg pain with activity

C. Headache

D. Dizziness (vertigo)

E. Bounding femoral pulses

F. Cyanosis of the lips at rest

Swipe for answer →

Answer 47

A, B, C, D

- **Nosebleeds**
- **Leg pain with activity**
- **Headache**
- **Dizziness (vertigo)**

Why: High pressure in the upper body causes headache, vertigo, and nosebleeds (even stroke); poor flow to the legs causes leg pain.

> 🧠 **"Head, Nose, Legs": high pressure up top (headache, nosebleed), low flow down below (leg pain).**

SINGLE ANSWER

What is the best screening action for coarctation?

A. Check oxygen saturation only

B. Compare pulses and BP in the arms and legs

C. Listen for a fixed split S2

D. Check capillary refill in one hand

Swipe for answer →

Answer 48

B

- **Compare pulses and BP in the arms and legs**

Why: Four-extremity pulse and BP comparison catches the arm-leg difference.

> 🧠 **"Check all 4 corners": both arms, both legs.**

SINGLE ANSWER

A 3-day-old with coarctation suddenly becomes pale, poorly perfused, and in shock. What is the most likely cause?

A. Dehydration from breastfeeding

B. Normal newborn transition

C. The ductus arteriosus is closing

D. The foramen ovale is opening

Swipe for answer →

Answer 49

C

- **The ductus arteriosus is closing**

Why: In severe neonatal CoA, the open ductus was supplying the lower body. When it closes, perfusion drops. PGE1 can reopen it.

> 🧠 **"Duct closes → baby crashes."**

SINGLE ANSWER

Which serious complication can result from uncorrected coarctation of the aorta?

A. Hearing loss

B. Coronary artery aneurysm

C. Mitral valve prolapse

D. Stroke (CVA)

Swipe for answer →

Answer 50

D

- **Stroke (CVA)**

Why: Chronic high blood pressure in the upper body can cause a CVA (listed on the addendum).

> 🧠 **"High pressure upstairs → head trouble": headache, nosebleed, stroke.**

SINGLE ANSWER

An 8-year-old's BP is 128/80 in the right arm and 90/60 in the leg. How should the nurse interpret this?

A. This is normal for a child

B. The cuff in the arm is too large

C. This suggests aortic stenosis

D. This suggests coarctation; normally leg BP is equal to or higher than arm BP

Swipe for answer →

Answer 51

D

- **This suggests coarctation; normally leg BP is equal to or higher than arm BP**

Why: In children the leg BP is normally the same or higher. A lower leg BP points to an obstruction (coarctation).

> 🧠 **"Legs should never lose to arms."**

SINGLE ANSWER

After coarctation repair, which finding needs the closest monitoring?

A. Visual acuity

B. Blood pressure (hypertension and recoarctation)

C. Blood glucose

D. Hearing

Swipe for answer →

Answer 52

B

- **Blood pressure (hypertension and recoarctation)**

Why: Older children may stay hypertensive after repair, and restenosis can occur. BP checks are lifelong.

> 🧠 **"Fix the coarct, then Follow BP forever."**

SELECT ALL THAT APPLY

A 5-day-old with critical coarctation is likely to show which findings?

A. Poor feeding

B. Tachypnea

C. Cool or mottled legs

D. Bounding femoral pulses

E. Weak femoral pulses

F. Hypertension in the legs

Swipe for answer →

Answer 53

A, B, C, E

- **Poor feeding**
- **Tachypnea**
- **Cool or mottled legs**
- **Weak femoral pulses**

Why: When the ductus closes, blood flow to the lower body drops → poor perfusion and heart failure. PGE1 reopens the ductus.

> 🧠 **"Duct closes → legs go cold → baby crashes."**

SINGLE ANSWER

A child with coarctation has frequent nosebleeds. Why?

A. Low platelets

B. Dry air only

C. High blood pressure in the upper body

D. A bleeding disorder caused by the aorta

Swipe for answer →

Answer 54

C

- **High blood pressure in the upper body**

Why: Pressure is high above the narrowing, causing headaches and nosebleeds.

> 🧠 **"High pressure upstairs → head trouble: headache, nosebleed, stroke."**

SINGLE ANSWER

Which syndrome is most associated with coarctation of the aorta?

A. Williams syndrome

B. Down syndrome

C. Turner syndrome

D. DiGeorge syndrome

Swipe for answer →

Answer 55

C

- **Turner syndrome**

Why: Turner (monosomy X): bicuspid aortic valve and coarctation. Down syndrome = AV canal.

> 🧠 **"Turner Turns the aorta."**

SINGLE ANSWER

In tricuspid atresia, how does blood leave the right atrium?

A. Through the mitral valve into the right ventricle

B. Through the foramen ovale into the left atrium

C. Through the ductus venosus

D. Through the tricuspid valve into the right ventricle

Swipe for answer →

Answer 56

B

- **Through the foramen ovale into the left atrium**

Why: The tricuspid valve is closed, so blood must cross the foramen ovale, then reach the lungs through a PDA (or VSD).

> 🧠 **"Front door locked? Take the side door" (the foramen ovale).**

SELECT ALL THAT APPLY

An infant with tricuspid atresia is at risk when the fetal shunts close. Which signs would the nurse expect?

A. Cyanosis

B. Tachycardia

C. Fixed split S2

D. Wide pulse pressure with bounding pulses

E. Dyspnea

Swipe for answer →

Answer 57

A, B, E

- **Cyanosis**
- **Tachycardia**
- **Dyspnea**

Why: The addendum: when these structures close, cyanosis, tachycardia, and dyspnea occur.

> 🧠 **"Shunts shut → Blue, Breathless, and Beating fast."**

SINGLE ANSWER

An infant with tricuspid atresia is receiving an IV prostaglandin E1 infusion. What is the nursing priority?

A. Check for hearing loss

B. Monitor for apnea

C. Restrict all oral fluids

D. Monitor for hypertension

Swipe for answer →

Answer 58

B

- **Monitor for apnea**

Why: PGE1 keeps the ductus open until surgery, but it can cause apnea. Continuous monitoring is required.

> 🧠 **"PGE1 = Patent ductus, Pause in breathing."**

SINGLE ANSWER

In tricuspid atresia, which chamber is underdeveloped?

A. Right ventricle

B. Left ventricle

C. None; all chambers are normal

D. Left atrium

Swipe for answer →

Answer 59

A

- **Right ventricle**

Why: No blood enters the right ventricle from the right atrium, so it stays small. The left ventricle does all the work.

> 🧠 **"No Tricuspid = Tiny right ventricle."**

SELECT ALL THAT APPLY

An infant with tricuspid atresia is on a prostaglandin E1 infusion. Which nursing actions are appropriate?

A. Monitor for apnea

B. Keep continuous pulse oximetry

C. Watch for fever and flushing

D. Keep the IV line patent

E. Stop the infusion once the saturation improves

F. Give it together with indomethacin

Swipe for answer →

Answer 60

A, B, C, D

- **Monitor for apnea**
- **Keep continuous pulse oximetry**
- **Watch for fever and flushing**
- **Keep the IV line patent**

Why: PGE1 keeps the ductus open until surgery. Side effects include apnea, fever, flushing, and low BP. Stopping it can close the ductus.

> 🧠 **"PGE1: Patent ductus, Pause in breathing, Pink flush, Pyrexia."**

SINGLE ANSWER

What is the long-term surgical plan for tricuspid atresia?

A. One-time VSD patch

B. Indomethacin only

C. Staged palliation: shunt, then Glenn (4–6 months), then Fontan (2–4 years)

D. Balloon valvuloplasty only

Swipe for answer →

Answer 61

C

- **Staged palliation: shunt, then Glenn (4–6 months), then Fontan (2–4 years)**

Why: Single-ventricle hearts are rerouted in stages so venous blood flows directly to the lungs.

> 🧠 **"Superior first, Inferior last: Glenn (SVC) then Fontan (IVC)."**

SELECT ALL THAT APPLY

Which are the four defects in Tetralogy of Fallot?

A. Overriding (dextroposed) aorta

B. Atrial septal defect

C. Coarctation of the aorta

D. Pulmonary stenosis

E. Right ventricular hypertrophy

F. Ventricular septal defect

Swipe for answer →

Answer 62

A, D, E, F

- **Overriding (dextroposed) aorta**
- **Pulmonary stenosis**
- **Right ventricular hypertrophy**
- **Ventricular septal defect**

Why: The addendum: PS, VSD, dextroposition of the aorta, and RV hypertrophy.

> 🧠 **"PROV": Pulmonary stenosis, RVH, Overriding aorta, VSD.**

SINGLE ANSWER

A child with TOF has polycythemia. The nurse understands this is:

A. An infection

B. Caused by too much oxygen

C. The body making more red blood cells because of low oxygen

D. A sign of iron deficiency

Swipe for answer →

Answer 63

C

- **The body making more red blood cells because of low oxygen**

Why: Chronic hypoxemia stimulates RBC production to carry more oxygen.

> 🧠 **"Low O2 → More Reds."**

SELECT ALL THAT APPLY

Which findings reflect chronic hypoxemia in uncorrected TOF?

A. Dyspnea

B. Clubbing of the fingers

C. High arm BP with weak femoral pulses

D. Cyanosis

E. Growth restriction

F. Polycythemia

G. Bounding pulses

Swipe for answer →

Answer 64

A, B, D, E, F

- **Dyspnea**
- **Clubbing of the fingers**
- **Cyanosis**
- **Growth restriction**
- **Polycythemia**

Why: The addendum's TOF signs: cyanosis, polycythemia, dyspnea, growth restriction, clubbing.

> 🧠 **"Blue, Clubbed, Thick blood, Small, and Short of breath."**

SINGLE ANSWER

An infant with TOF suddenly becomes very blue and breathes fast while crying. What should the nurse do FIRST?

A. Lay the infant flat on the back

B. Give albuterol

C. Feed the infant to calm them

D. Place the infant in a knee-chest position

Swipe for answer →

Answer 65

D

- **Place the infant in a knee-chest position**

Why: Knee-chest raises systemic resistance, which pushes more blood into the lungs. Then calm the infant, give O2, and get emergency help.

> 🧠 **"Tet spell? Tuck like a frog."**

SINGLE ANSWER

Why does a child with uncorrected TOF squat after playing?

A. Squatting rests tired leg muscles

B. Squatting increases systemic resistance and sends more blood to the lungs

C. Squatting lowers blood pressure

D. Squatting closes the VSD

Swipe for answer →

Answer 66

B

- **Squatting increases systemic resistance and sends more blood to the lungs**

Why: Squatting kinks the leg arteries, raising systemic resistance so less blood crosses the VSD and more goes to the lungs.

> 🧠 **"Squat = Squeeze the legs, Send blood to the lungs."**

SINGLE ANSWER

A child with TOF and polycythemia has vomiting and diarrhea. Why is this a concern?

A. It lowers the red blood cell count

B. It is not a concern

C. Dehydration thickens the blood and raises the risk of clots and stroke

D. It causes the VSD to close

Swipe for answer →

Answer 67

C

- **Dehydration thickens the blood and raises the risk of clots and stroke**

Why: Polycythemic blood is already thick. Dehydration makes it thicker, raising the risk of thrombosis or stroke. Keep the child hydrated.

> 🧠 **"Thick blood + dry = clot risk": keep them hydrated.**

SINGLE ANSWER

Which finding indicates LONG-STANDING hypoxemia in a child with TOF?

A. Clubbing of the fingers

B. Pale, pink nail beds

C. Hypertension in the arms

D. Bounding pulses

Swipe for answer →

Answer 68

A

- **Clubbing of the fingers**

Why: Clubbing develops after months of low oxygen. It's on the addendum's TOF list.

> 🧠 **"Clubs take time: months of blue."**

SINGLE ANSWER

A child with uncorrected TOF has a hemoglobin of 20 g/dL and a hematocrit of 62%. What is the priority teaching?

A. Start iron supplements

B. Keep the child well hydrated, especially during illness or hot weather

C. No teaching is needed; these values are normal

D. Restrict fluids to protect the heart

Swipe for answer →

Answer 69

B

- **Keep the child well hydrated, especially during illness or hot weather**

Why: This is compensatory polycythemia. Thick blood plus dehydration raises the risk of clots and stroke.

> 🧠 **"Thick blood + dry = clot risk: keep them drinking."**

SINGLE ANSWER

Which chest x-ray finding is classic for TOF?

A. Boot-shaped heart with decreased pulmonary vascular markings

B. Egg-shaped heart

C. Rib notching

D. Enlarged heart with increased pulmonary markings

Swipe for answer →

Answer 70

A

- **Boot-shaped heart with decreased pulmonary vascular markings**

Why: Pulmonary stenosis means less blood reaches the lungs (fewer markings), and RVH tips the apex up into a boot shape.

> 🧠 **"Boot = TOF, Egg = TGA, Notches = Coarct."**

SELECT ALL THAT APPLY

What should the nurse teach the parents of an infant with TOF about tet spells?

A. Place the infant in a knee-chest position

B. Call emergency services if the spell doesn't resolve quickly

C. Avoid long periods of crying

D. Lay the infant flat on the back

E. Give a large bottle right away

F. Keep the infant calm

Swipe for answer →

Answer 71

A, B, C, F

- **Place the infant in a knee-chest position**
- **Call emergency services if the spell doesn't resolve quickly**
- **Avoid long periods of crying**
- **Keep the infant calm**

Why: Knee-chest raises systemic resistance, pushing more blood to the lungs. Prolonged spells can lead to syncope or worse.

> 🧠 **"Tet spell? Tuck like a frog, then call for help."**

SINGLE ANSWER

Why does the amount of cyanosis vary among infants with TOF?

A. It depends on the infant's blood type

B. It depends on the room temperature only

C. It depends on how severe the pulmonary stenosis is

D. It depends on the size of the ASD

Swipe for answer →

Answer 72

C

- **It depends on how severe the pulmonary stenosis is**

Why: More obstruction → more blood crosses the VSD right-to-left → more cyanosis. Mild obstruction = "pink tet."

> 🧠 **"More Pulmonary stenosis = More Purple."**

SELECT ALL THAT APPLY

Which are signs of RIGHT-sided heart failure?

A. Increased venous pressure

B. Dyspnea as the first sign

C. Edema/weight gain from fluid

D. Hepatomegaly

E. Crackles in the lungs

Swipe for answer →

Answer 73

A, C, D

- **Increased venous pressure**
- **Edema/weight gain from fluid**
- **Hepatomegaly**

Why: Right-sided failure backs fluid up into the body (liver, veins). Left-sided failure backs up into the LUNGS (crackles, dyspnea).

> 🧠 **"Left = Lungs. Right = the Rest of the body" (liver).**

SINGLE ANSWER

Which finding indicates LEFT-sided heart failure?

A. Hepatomegaly

B. Jugular venous distention

C. Ascites

D. Crackles (rales) in the lungs

Swipe for answer →

Answer 74

D

- **Crackles (rales) in the lungs**

Why: Left-sided failure → fluid backs up into the lungs: dyspnea, crackles, cyanosis. It eventually causes right-sided failure too.

> 🧠 **"Left = Lungs."**

SELECT ALL THAT APPLY

Which interventions reduce the heart's workload in a child with heart failure?

A. Increasing fluid intake

B. Small, frequent feedings with rest periods

C. Vasodilators

D. Large feedings every 6 hours

E. Inotropic medications such as digoxin

F. Diuretics

Swipe for answer →

Answer 75

B, C, E, F

- **Small, frequent feedings with rest periods**
- **Vasodilators**
- **Inotropic medications such as digoxin**
- **Diuretics**

Why: The addendum: reduce workload with diuretics, inotropes, and vasodilators. Small frequent feeds save energy.

> 🧠 **"Dump, Pump, Relax": Diuretics dump fluid, Inotropes help it pump, Vasodilators relax the vessels.**

SINGLE ANSWER

What are the EARLIEST signs of heart failure in an infant?

A. Bradycardia and hypertension

B. Ankle edema

C. Tachycardia and tachypnea

D. Clubbing

Swipe for answer →

Answer 76

C

- **Tachycardia and tachypnea**

Why: The addendum lists tachycardia and tachypnea first. Infants show HF through fast breathing, fast heart rate, and feeding trouble before edema.

> 🧠 **"Fast and Fast": fast heart, fast breathing.**

SINGLE ANSWER

Before giving digoxin to an infant, what should the nurse do?

A. Give it with a full bottle of formula

B. Count the apical pulse for 1 full minute and hold if below the prescribed limit

C. Check the blood pressure only

D. Take the radial pulse for 15 seconds

Swipe for answer →

Answer 77

B

- **Count the apical pulse for 1 full minute and hold if below the prescribed limit**

Why: Digoxin slows the heart. An apical pulse for a full minute is required; hold and notify if below parameters (often under 90–110 in infants).

> 🧠 **"Dig: count a full minute, then Don't go if it's low."**

SINGLE ANSWER

A child receives furosemide and digoxin. Which lab value is the priority to monitor?

A. Calcium

B. Potassium

C. Hemoglobin A1c

D. Glucose

Swipe for answer →

Answer 78

B

- **Potassium**

Why: Furosemide wastes potassium, and low potassium increases the risk of digoxin toxicity.

> 🧠 **"Lasix Loses K → Dig gets Dangerous."**

SINGLE ANSWER

An infant on digoxin is vomiting and has a heart rate of 80. What should the nurse suspect?

A. Normal side effects; give the next dose

B. Digoxin toxicity: hold the dose and notify the provider

C. A viral infection only

D. Dehydration from overfeeding

Swipe for answer →

Answer 79

B

- **Digoxin toxicity: hold the dose and notify the provider**

Why: Early digoxin toxicity in infants: vomiting, poor feeding, and bradycardia. Low potassium increases the risk.

> 🧠 **"Dig toxic = Dizzy, Dumping (vomiting), Dropping heart rate."**

SINGLE ANSWER

What is the BEST way for parents to monitor fluid status at home in an infant on furosemide?

A. Check the color of the stool

B. Daily weight on the same scale, at the same time

C. Count breaths once a week

D. Measure the head circumference

Swipe for answer →

Answer 80

B

- **Daily weight on the same scale, at the same time**

Why: A rapid weight gain means fluid is building up. Also count wet diapers.

> 🧠 **"Same Scale, Same time, Same clothes."**

SELECT ALL THAT APPLY

Which feeding strategies help an infant with heart failure?

A. Diluting the formula

B. Small, frequent feedings

C. Limiting each feed to about 20–30 minutes, then gavage the rest if ordered

D. Feeding in a semi-upright position

E. Large feedings every 6 hours

F. A soft nipple with a larger opening

Swipe for answer →

Answer 81

B, C, D, F

- **Small, frequent feedings**
- **Limiting each feed to about 20–30 minutes, then gavage the rest if ordered**
- **Feeding in a semi-upright position**
- **A soft nipple with a larger opening**

Why: These save the infant's energy while maximizing calories.

> 🧠 **"Small, Soft, Sitting up, Stop at 30."**

SINGLE ANSWER

Before giving captopril (an ACE inhibitor) to an infant with heart failure, which assessment is the priority?

A. Blood pressure

B. Head circumference

C. Temperature

D. Blood glucose

Swipe for answer →

Answer 82

A

- **Blood pressure**

Why: ACE inhibitors dilate vessels to reduce afterload and can cause hypotension. They can also raise potassium.

> 🧠 **"ACE = Afterload Cut, check the pressure."**

SINGLE ANSWER

Which position is best for an infant with heart failure and tachypnea?

A. Trendelenburg (head down)

B. Prone with the head lower than the feet

C. Flat on the back

D. Semi-upright (semi-Fowler)

Swipe for answer →

Answer 83

D

- **Semi-upright (semi-Fowler)**

Why: Upright positioning lets the lungs expand and decreases the work of breathing.

> 🧠 **"Heads up for a tired heart."**

SELECT ALL THAT APPLY

Which actions reduce the heart's oxygen demand in an infant with heart failure?

A. Keep a neutral thermal environment

B. Keep the room cold

C. Minimize prolonged crying

D. Cluster care to allow rest

E. Treat fever promptly

F. Wake the infant often to stimulate

Swipe for answer →

Answer 84

A, C, D, E

- **Keep a neutral thermal environment**
- **Minimize prolonged crying**
- **Cluster care to allow rest**
- **Treat fever promptly**

Why: Cold, fever, crying, and frequent handling all increase the heart's workload.

> 🧠 **"Rest, Warm, Cool fever, Calm."**

SELECT ALL THAT APPLY

Which findings are consistent with rheumatic fever?

A. Chorea

B. Peeling of the fingertips

C. Macular rash on the trunk (erythema marginatum)

D. Strawberry tongue

E. New systolic murmur

F. Swollen, tender joints

G. Subcutaneous nodules

Swipe for answer →

Answer 85

A, C, E, F, G

- **Chorea**
- **Macular rash on the trunk (erythema marginatum)**
- **New systolic murmur**
- **Swollen, tender joints**
- **Subcutaneous nodules**

Why: These are the Jones major criteria from the addendum. Strawberry tongue and peeling point to Kawasaki.

> 🧠 **"JONES": Joints, O (♥ carditis), Nodules, Erythema marginatum, Sydenham chorea.**

SINGLE ANSWER

Which lab result shows a recent strep infection in suspected rheumatic fever?

A. Elevated troponin only

B. Positive blood culture for Staph

C. Low platelet count

D. Elevated ASO titer

Swipe for answer →

Answer 86

D

- **Elevated ASO titer**

Why: The addendum: positive ASO titer plus increased ESR and CRP. ASO shows PRIOR strep; it doesn't diagnose RF by itself.

> 🧠 **"ASO = Already Strep Occurred."**

SINGLE ANSWER

The nurse notes chorea in a child with rheumatic fever. Which describes chorea?

A. Sudden, involuntary, purposeless movements of the limbs

B. Swelling of the large joints

C. A painless rash on the trunk

D. Firm nodules over tendons

Swipe for answer →

Answer 87

A

- **Sudden, involuntary, purposeless movements of the limbs**

Why: Sydenham chorea = jerky involuntary movements. It may appear later than other signs.

> 🧠 **"Chorea = Choreography gone wild" (dancing that the child can't control).**

SINGLE ANSWER

What determines the long-term prognosis of rheumatic fever?

A. How high the fever got

B. How much the heart (especially the valves) is involved

C. How many joints were swollen

D. Whether the rash was itchy

Swipe for answer →

Answer 88

B

- **How much the heart (especially the valves) is involved**

Why: Arthritis heals without damage; carditis can leave permanent valve disease (mitral/aortic).

> 🧠 **"Joints heal, the Heart remembers." Rheumatic = Valves.**

SELECT ALL THAT APPLY

Which are part of managing acute rheumatic fever?

A. Ongoing prophylaxis to prevent recurrence

B. IVIG as first-line therapy

C. Antibiotics to eliminate group A strep

D. Bed rest during the acute phase

E. Ear tubes

Swipe for answer →

Answer 89

A, C, D

- **Ongoing prophylaxis to prevent recurrence**
- **Antibiotics to eliminate group A strep**
- **Bed rest during the acute phase**

Why: The addendum: bed rest and antibiotics to eliminate GABHS. Secondary prophylaxis (e.g., benzathine penicillin) prevents recurrence.

> 🧠 **"Kill the strep, Rest the heart, Prevent the repeat."**

SINGLE ANSWER

A child with rheumatic fever has painful, swollen joints. Which treatment helps the joint inflammation?

A. Ice baths every hour

B. IVIG

C. An anti-inflammatory such as aspirin or another NSAID, as prescribed

D. Indomethacin to close the ductus

Swipe for answer →

Answer 90

C

- **An anti-inflammatory such as aspirin or another NSAID, as prescribed**

Why: Anti-inflammatory therapy treats the arthritis; antibiotics eradicate the strep. Joints heal without lasting damage.

> 🧠 **"Joints heal, the Heart remembers."**

SINGLE ANSWER

What is the purpose of regular benzathine penicillin injections after rheumatic fever?

A. To lower the platelet count

B. To prevent repeat strep infections that could cause more heart valve damage

C. To treat the chorea

D. To close a VSD

Swipe for answer →

Answer 91

B

- **To prevent repeat strep infections that could cause more heart valve damage**

Why: Secondary prophylaxis prevents recurrences. Each attack can worsen valve damage.

> 🧠 **"Prevent the repeat, protect the valves."**

SELECT ALL THAT APPLY

Which nursing actions are appropriate for a child with Sydenham chorea?

A. Reassure the family that the movements are temporary

B. Reduce stress

C. Tell the child to try harder to hold still

D. Help with eating and writing

E. Restrain the child's arms

F. Protect from injury, for example with padded side rails

Swipe for answer →

Answer 92

A, B, D, F

- **Reassure the family that the movements are temporary**
- **Reduce stress**
- **Help with eating and writing**
- **Protect from injury, for example with padded side rails**

Why: The movements are involuntary and get worse with stress. They gradually resolve.

> 🧠 **"Chorea care: Safe, Help, Reassure, Calm."**

SELECT ALL THAT APPLY

Which are EARLY signs of Kawasaki disease?

A. Enlarged cervical lymph nodes

B. Peeling skin on the fingers

C. Red eyes (conjunctival injection) without discharge

D. High fever that doesn't respond to antipyretics

E. Coronary artery aneurysm

F. Strawberry tongue and red, cracked lips

G. Swollen hands and feet

Swipe for answer →

Answer 93

A, C, D, F, G

- **Enlarged cervical lymph nodes**
- **Red eyes (conjunctival injection) without discharge**
- **High fever that doesn't respond to antipyretics**
- **Strawberry tongue and red, cracked lips**
- **Swollen hands and feet**

Why: Peeling (desquamation) and aneurysms are LATE findings on the addendum.

> 🧠 **"CRASH and BURN": Conjunctivitis, Rash, Adenopathy, Strawberry tongue, Hands/feet + BURN (fever for 5+ days).**

SELECT ALL THAT APPLY

Which are LATE findings in Kawasaki disease?

A. Coronary artery aneurysm

B. Onset of high fever

C. Peeling skin (desquamation)

D. Rising platelet count

E. Strawberry tongue appearing for the first time

Swipe for answer →

Answer 94

A, C, D

- **Coronary artery aneurysm**
- **Peeling skin (desquamation)**
- **Rising platelet count**

Why: The addendum's late signs: desquamation, increased platelets, aneurysms. High platelets raise the clot risk in aneurysms.

> 🧠 **"Late = Peel, Platelets, Pouches" (aneurysms are pouches).**

SINGLE ANSWER

What is the main purpose of IV immunoglobulin (IVIG) in Kawasaki disease?

A. To decrease the immune response and lower the risk of coronary aneurysms

B. To raise the platelet count

C. To kill the bacteria causing the disease

D. To close the ductus arteriosus

Swipe for answer →

Answer 95

A

- **To decrease the immune response and lower the risk of coronary aneurysms**

Why: IVIG calms the inflammation; given early (within about 10 days) it reduces coronary damage. NOTE: your addendum lists ibuprofen for inflammation, but your integrated study guide and current AHA guidance use ASPIRIN with IVIG. Ibuprofen can block aspirin's anti-clotting effect. Ask your professor which answer they expect.

> 🧠 **"Kawasaki Crowns the Coronaries." IVIG protects the crowns.**

SINGLE ANSWER

Which follow-up is MOST important after Kawasaki disease?

A. Serial echocardiograms to check the coronary arteries

B. Yearly hearing tests

C. Monthly throat cultures

D. Daily blood glucose checks

Swipe for answer →

Answer 96

A

- **Serial echocardiograms to check the coronary arteries**

Why: The big risk is coronary aneurysm, which echo detects. Most children recover fully, but some need heart surgery.

> 🧠 **"Echo the coronaries."**

SINGLE ANSWER

A child received IVIG for Kawasaki disease. When can the MMR and varicella vaccines be given?

A. 11 months after IVIG

B. 1 month after IVIG

C. Never

D. Right away

Swipe for answer →

Answer 97

A

- **11 months after IVIG**

Why: IVIG's antibodies block the response to live vaccines. Non-live vaccines don't need to wait.

> 🧠 **"2 grams → wait 11 months."**

SINGLE ANSWER

Which description of the FEVER in Kawasaki disease is correct?

A. Fever only at night

B. Fever that breaks with one dose of acetaminophen

C. Low-grade fever for 1 day

D. High fever for 5 or more days that doesn't respond to antipyretics or antibiotics

Swipe for answer →

Answer 98

D

- **High fever for 5 or more days that doesn't respond to antipyretics or antibiotics**

Why: The addendum: high fever that doesn't respond to therapy. Children are also extremely irritable.

> 🧠 **"BURN for 5: the fever that won't quit."**

SINGLE ANSWER

A child with Kawasaki disease is taking aspirin. Which teaching is essential?

A. Start ibuprofen along with the aspirin

B. Report exposure to flu or chickenpox, and get the flu vaccine (Reye syndrome risk)

C. Aspirin can be stopped whenever the child feels better

D. Give extra aspirin for any fever

Swipe for answer →

Answer 99

B

- **Report exposure to flu or chickenpox, and get the flu vaccine (Reye syndrome risk)**

Why: Aspirin plus influenza or varicella can cause Reye syndrome. Ibuprofen can block aspirin's antiplatelet effect. (Note: your addendum slide lists ibuprofen; your study guide and AHA say aspirin.)

> 🧠 **"Aspirin kid: no Flu, no Pox."**

SELECT ALL THAT APPLY

A child is in the late (convalescent) phase of Kawasaki disease. Which teaching is correct?

A. Follow-up can stop once the fever is gone

B. Keep all follow-up echocardiograms

C. Peeling of the fingers and toes is expected

D. Peeling means a new infection

E. Platelets rise in this phase, increasing the clot risk

Swipe for answer →

Answer 100

B, C, E

- **Keep all follow-up echocardiograms**
- **Peeling of the fingers and toes is expected**
- **Platelets rise in this phase, increasing the clot risk**

Why: Late phase = peeling, thrombocytosis, and possible aneurysms. Echo follow-up continues.

> 🧠 **"Late = Peel, Platelets, Pouches (aneurysms)."**